

RIVER OAKS CHILDREN'S MEDICAL CENTER

Division of Pediatric Neuro-Oncology | 1840 Westbridge Avenue, Sacramento, CA 95816 | (555) 014-2280

FICTIONAL SAMPLE - NOT A REAL MEDICAL RECORD

Date: September 29, 2026	Patient: Emily Carter
DOB: March 14, 2018	MRN: FICT-204817
Referring Physician: Karen Liu, MD	Visit Type: New diagnosis consultation

RE: Newly Diagnosed Medulloblastoma

Dear Dr. Liu,

Thank you for referring Emily Carter, an 8-year-old girl who was evaluated after approximately four weeks of progressive morning headaches, intermittent vomiting, gait instability, and increasing fatigue. Neurologic examination was notable for mild truncal ataxia and difficulty with tandem gait, without focal motor deficit.

Diagnostic findings. Brain MRI demonstrated a 3.6 cm midline posterior fossa mass centered in the cerebellar vermis with heterogeneous enhancement and associated obstructive hydrocephalus. MRI of the spine showed no definite evidence of leptomeningeal dissemination. She subsequently underwent suboccipital craniotomy with near-total resection on September 25, 2026. Postoperative MRI showed a small residual area measuring less than 1.0 cm².

Pathology. Histopathologic examination is consistent with medulloblastoma. Molecular characterization is in progress, including subgroup classification and additional risk-stratifying studies. Final integrated pathology will be reviewed at our multidisciplinary pediatric neuro-oncology tumor board.

Assessment. Newly diagnosed pediatric medulloblastoma, status post near-total surgical resection. At present there is no radiographic evidence of spinal metastatic disease. Definitive risk classification will depend on the final molecular pathology, postoperative imaging, and cerebrospinal fluid staging when clinically appropriate.

Plan. We discussed the diagnosis and next steps with Emily's parents. The immediate plan is postoperative recovery with neurosurgery, completion of staging, review of final molecular results, and multidisciplinary treatment planning. Standard treatment commonly includes risk-adapted craniospinal irradiation and systemic chemotherapy; the exact regimen will be determined after complete staging and discussion with the family. Supportive care referrals, including physical therapy, audiology baseline testing, and psychosocial support, have also been initiated.

Emily is clinically stable following surgery. We will contact the family after tumor board review and coordinate ongoing care with your office. Please feel free to contact our team with any questions.

Sincerely,

Jonathan M. Hayes, MD
Pediatric Neuro-Oncology
River Oaks Children's Medical Center
NPI: SAMPLE-NOT-VALID

This document is entirely fictional and was created for demonstration/testing purposes. All names, identifiers, institutions, contact details, and clinical details are invented.